جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : اسماء محمد الضمراني حسانين
المحافظة : الجيزه **الحي / المدينة :** امبابه **العنوان :** 0
رقم البطاقة : 28005042103605 **رقم التليفون :** 01226285942 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 04-05-1980 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: اسماء محمد الضمراني حسانين
National ID: 28005042103605

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

01/27/2026

MDT FORM

Name:	Asmaa Mohamed Eldamarany Hassanien	**MRN:**	6607
Age:	46 Y	**ID:**	3418/25
Menopausal Sta	PRE	**Phone:**	01143505069
NRS SCORE:	0	**Marital Status:**	MARRIED
BMI:	25	**Family History:**	
CO Morbidity:	FMF ON COLICHICINE,,+VE FH	**Reason Of MDT:**	NEW CASE

Clinical Data:

BX: IDC G 3

ER: 0/8 **PR:** 3/8 **HER2:** +1 **KI67:** 70

Sonomammography:
CESM-RT UPPER CENTRAL MASS WITH SUSPECTED LINEAR DUCT EXTENSION
SINGLE PATHOLOGICAL LN CT2 N1 M0

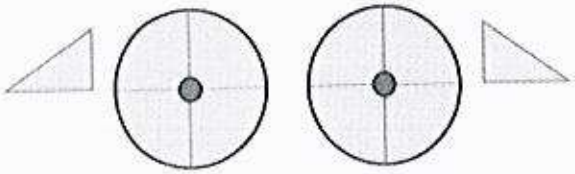
Mets Workup:
CT CAP FREE
BONE SCAN FREE

History:

Recommendations:
FOR BRACKTING OF MASS AND DILATED DUCT WITH CLIPS
NACT THEN RT WLE AND SLNB(ACCORDING TO TARGETED US)

MDT Outcome & Plan
For Bracketing by clips
Start NACT (pembrolizumab + paclitaxel /carboplatin X 4 cycles then pembrolizumab + AC X 4 cycles) then Rt WLE + SLNB

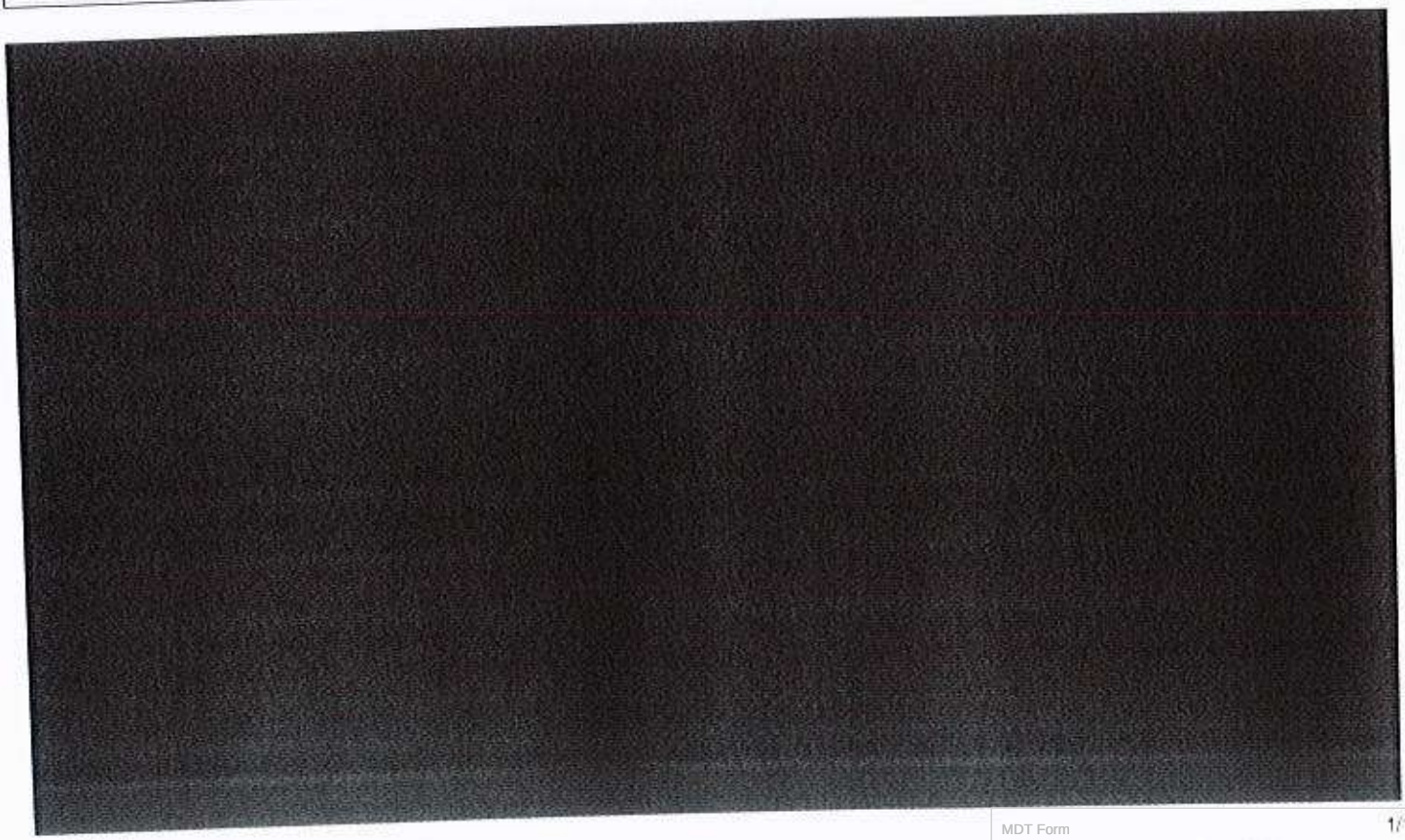
R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



جمهورية مصر العربية
بطاقة تحقيق الشخصية



اسماء
محمد الضمرانى حسانين
٢ ش الجمهورية - المنيرة
امبابة - الجيزه

٢٨٠٠٥٠٤٢١٠٣٦٠٥

١٩٨٠/٠

JR0165554

٢٨٠٠٥٠٤٢١٠٣٦٠٥ ٢٠٢٢/١١
ربة منزل

انثى مسلمة

١ البطاقة سارية حتى ٢٠٢٩/١١/٢٣

محمود عادل
DMS ٤٤٠٤٦١٤١٤

عادل كيلاوى

Name: اسماء محمد الضمراني حسانين	
MR. : 6607	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

wt 90 ht 167 sa 2 prem rt breast cancer ct2 n1 mo idc 3 er 0/8 pr3/8 her2-ve ki 70 clips inserted cts iliac osseous lesions bs free ef 65% 7/4/2026 for c1 keytruda 200 w1 taxol 160 carboplatin auc 2 neoadj w2 ,3 ph

clinical pharmacy note : foe w2 taxol 150 carbo auc 2 ... patient didm't recieve keytruda pending approval

C4 taxol 150 carbo auc 2 ... patient didm't recieve keytruda pending approval

on exam rt breast mass 2x3 cm . sm partial radiological response bs 6/2026 free for w5 taxol 150 carbopl auc 2 for c1 keytruda 200 after approval mnot yet approved w6 7 ph

Pharmacy Visit Wt: 90, Ht: 167, BSA: 2, AdBwt: 71.2 For W6 Taxol 150 + Carboplatin AUC 2 Labs 22/6 ... accepted

for w7 taxol 150 carbopl auc 2 .no approval of keytruda for referral w8 ph alt 79 for liver support

clinical pharmacy note : w8 taxol 150 mg carbo auc 2

Pharmacy Visit Last week in 20/7 wasn't given due to delayed approvals Wt: 90, Ht: 167, BSA: 2, AdBwt: 71.2 For W8 Taxol 150 + Carboplatin AUC 2 LAbs 13/7 ... accepted

on exam rt 12 o clock mass 2x2 for w9 taxol 150 carbopl auc 2 neoadj .w10 11 ph alt 58

clinical pharmacy note : w11 taxol 150 mg carbo auc2

cycles miscounted, pt finished 12 weeks of TC, keytruda not approved by MOH nor by NGO, to start DD AC next week

on exam rt 12 oclock mass 2x2 rt ax ln 1x2 mobile for c1 DD ac adria 120 endoxan 1200 neoadj and 3 proh neupogen

تحريرا في ,

التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : **dina makram**

يعتمد ,

STAVE/ ROUSSY INTERNATIONAL AGENCY

Patient Name	: الأستاذه / اسماء محمد الضمراني	Request No.	: 13000981
Patient No.	: 28005042103605	Request Date	: 03/11/2025
Path No.	: 3351/25	Verification Date	: 19/11/2025
Sex	: Female	Financial Category	: جوستاف روسي - مباشره
Age	: 45 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast 12 o'clock mass, BIRADs 4c.
Nature of Specimen: Tru-cut biopsy, collected on 02/11/2025.

Gross Examination:
- Three fragmented grey white tissue cores, the longest was 1.5 cm. Totally submitted.
Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for:
- GATA3 clone (L50-823)□, ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
* DCIS: Absent.
* Angioinvasion: Absent.
* Perineural invasion: Absent.

* IPT (Positive control showed appropriate reactivity):
- GATA-3: Diffuse strong posistive staining in tumor cells.
- ER: Negative nuclear staining in tumor cells (score 0/8).
- PR: Weak nuclear staining in 5% of tumor cells (score 3/8).
- Her2: Faint incomplete membrane staining in more than 10% of tumor cells (score 1+).
- Ki67%: 70%.

Diagnosis:
Right breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 3.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

ISTAVE/ KOUSSY INTERNATIONAL

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Age	: 45 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

IPT:
- GATA-3: Diffuse strong posistive staining in tumor cells.
- ER: Negative (score 0/8).
- PR: Positive (score 3/8).
- Her2: Low (score 1+).
- Ki67%: 70%.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY
INTERNATIONAL

Name : Asmaa Mohamed Al Damarany
Branch : Portable
Ref.Physician Prof :

Date : 20-12-2025 02:48
Medical No. : GRI-133415
Ins.Company :

- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Right renal simple cortical cyst 1x1.5cm.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Hetereogenous both iliac bones with few scattered lytic lesions for bone scan assessment.

Opinion:

- **No hepatic metastasis.**
- **Hetereogenous both iliac bones with few scattered lytic lesions for bone scan assessment.**

Dr. Mai Shaaban MD

Dr. Mai Shaaban

Dr. Mai Shaaban

Name: Asmaa Mohamed El Domrani
Date: 2025-12-18

BILATERAL DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH RESOLUTION BREAST SONOGRAPHY REVEALED:

Clinical summary:
A known case of right breast carcinoma

FINDINGS:

LEFT BREAST:

- Heterogenous dense breast parenchyma, (ACR c).
- No spiculations, masses asymmetry , distortion or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary nodes.

RIGHT BREAST:

- Heterogenous dense breast parenchyma, (ACR c).
- An UOQ ill defined lesion is seen.
- Ultrasound demonstrated a 11 o'clock, about 3 cm from nipple and 3.5mm deep to the skin, ill defined heterogenous hypoechoic solid mass with mild internal vascularity on color Doppler study, measuring 38x22mm.
- Normal skin/nipple/areola complex.
- A single axillary node is seen.
- Free infra and supra-clavicular regions

CONCLUSION:
Left breast (BIRADS 1): normal study.
Right breast (BIRADS 6): UOQ malignant mass.

Best regards,
Dr. Mahi Mokhtar

Patient Name: asmaa mohamed
Date: 5/8/2025

BILATERAL SONOGRAPHY

- Normal fibro-glandular parenchyma is seen, (ACR-C).
- The left breast shows retro-areolar medium density mass with partially defined partially obscured margins.
- No suspicious clustered micro-calcification is seen.
- Normal skin thickness, subcutaneous fat and nipple areolar complex.

COMPLEMENTARY BREAST ULTRASOUND:-.

- Bilateral breasts are of heterogeneous parenchymal texture, mostly glandular.
- The right breast shows well defined hypo-echoic solid mass with macrolobulated margin, seen at the retro-areolar region showing heterogeneous texture with anechoic areas of cystic changes and hyper echoic areas as well. The lesion is measuring about 2.7x1.5x2.0 cm in dimensions. By Doppler study it shows internal vascularity. By Micorpure technique its shows calcifications.
- No dilated lactiferous ducts.
- Normal skin thickness & subcutaneous fat lobules
- Both axillae show enlarged LNs with preserved echogenic fatty hilum.

OPINION:

- right breast suspicious looking lesion as described. Histopathological assessment is recommended (BIRADS IV a)

DR. walaa alwasl, MSc.

Name: Asmaa Mohamed El Domrani
Date: 2025-12-18

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Best regards,
Dr. Mahi Mokhtar

GUSTAVE ROUSSY
INTERNATIONAL

Name : Asmaa Mohamed Al Damarany
Branch : Portable
Ref.Physician Prof :

Date : 20-12-2025 02:48
Medical No. : GRI-133415
Ins.Company :

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Clinical History:

- **No available history?! For metastatic work up.**

Findings:

- Middle lobe peripherally located tiny pulmonary nodule is seen measuring 3mm.
- Clear other both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.

OPINION:

- Middle lobe peripherally located tiny pulmonary nodule for follow up.

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION: -

Findings:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.

Women's Health Outreach Program

Name: Asmaa Mohamed
Date: 12/1/2026

Contrast Enhanced Digital Mammography

Clinical summary: 45 years old female with IDC III

Technique

MLO and CC views (low energy & recombined post contrast)
Targeted High resolution ultrasound

Detailed Findings:

Density: ACR C (heterogeneous dense)
Background Uptake: mild nodular

Right breast

- Low energy (non contrast images) show : upper central middle third spiculated hyperdense mass .
- Recombined (post contrast) images show: upper central intense heterogonous enhancing mass lesion (2x3x2.4cm) with suspected faint linear non mass enhancement seen extend from the lesion to the retroareaolar region appreciated at MLO view measures roughly (4x1.2 cm) , with small nodule located at about 5cm from index lesion 2 cm from nipple seem only at MLO view .
- Ultrasound :11 :00 o clock heterogonous mass measured about 3.8x2.2 cm about 3 cm from nipple with anterior related area of altered echogenicity, dilated ducts showing echogenicities within.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Single pathological lymph node level one detected measured about 16 mm with focal cortical thickness reached about 4 mm , no extracapsular extension.

Left breast

- Low energy (non contrast images) show : no masses, asymmetry, distortion or suspicious calcifications.
- Recombined (post contrast) images show: no pathological uptake.
- Ultrasound demonstrated no solid or cystic masses.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

Right breast : upper central pathologically proven malignant enhancing mass lesion with suspected anterior intraductal extension **(BIRADS 6): left breast (BIRADS 2):** Normal study

DR. Marwa abdelhakim ,msc .
DR.Lamiaa Adel ,MD

Nuclear Medicine Unit وحدة الطب النووى

Asmaa Mohamed El Damarany ***Date: 20/01/2026***

Tc99m MDP SKELETAL SCINTIGRAPHY

History

- A 45-year-old female with history of breast cancer.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- No definite evidence of bone secondaries could be seen.
- Features suggestive of degenerative changes of the spine and big joints.

IMPRESSION

- The skeleton is scintigraphically free from metastases.

Appreciated,

Dr.Hossam Sekkina, M.Sc.

Dr. Alaa Nagi, MD
Nuclear Medicine